



**Claims PHD:
Pretty Hard Decisions**

Dr Anthony Michalski

- Cancer in children is mercifully rare; only 1:600 children will get a malignancy by the time they are 15 years of age.
- Cancer in children is the second commonest cause of death in childhood.
- Children develop different malignancies to adults and the type of tumour they develop are related to age.
- The nomenclature of childhood tumours is different—the classical adult classifications of benign and malignant and tumours stage (TNM) are not used in paediatrics. This can lead to arguments as to what constitutes a malignancy in insurance v clinical terms.
- The advent of molecular biology has led to a massive improvement in the understanding of childhood malignancy. Many cancers are now defined by their biological phenotype.
- Traditionally, around 5% of cancers were thought to be caused by an inherited predisposition. We now know that in some the inheritance risk is much higher and over 8% of children will carry genetic mutations that may have implications for them or for the risk of cancer in their relatives.
- Conventional therapy (chemotherapy and radiation) leads to late effects which can predispose to illness in the future.
- Novel therapies block the molecular drivers of cancer but turn it into a chronic illness. The drugs are extremely expensive and have to be taken for very long periods of time (potentially for life). The health economics need to be addressed. No real data on the very long outcomes and toxicities of molecularly targeted therapies.

Child CI—Case Study 1

Policy definition - Policy Start Date 01/11/07

- A) A “relevant child” means a natural child, legally adopted child or stepchild of the Life Assured, while such a child is aged more than 26 weeks and less than 18 years during the term of the policy
- B) On receipt of proof of diagnosis before the Expiry date of a Critical illness of a Relevant Child, and providing the Relevant Child survives for at least 28 days from the diagnosis, there shall become payable the lesser of 50% of the sum assured or £15,000 per Relevant Child in aggregate in respect of all claims arising under all policies on which the Life Assured is the Parent, or Step Parent of a Relevant Child. Payment of the benefit will not affect the sum assured
- C) For a Relevant Child the definition of Critical Illness does not include Permanent and total Disability
- D) No claims will be payable in respect of a Relevant Child as a direct or indirect result of congenital, hereditary or pre-existing conditions. A pre-existing condition is any medical condition, disorder or handicap from which a child is already suffering before the latest of:
- the date the policy is issued
 - the 26th week after the child's birth
 - for legally adopted children, the date of adoption
- E) Only one claim will be paid in respect of each Relevant Child up to a maximum of two claims in total for all Relevant Children
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- H) Where diagnosis of a Critical Illness takes place before the Expiry Date, but the survival period expires after the Expiry Date, the sum assured shall become payable provided the Relevant Child survives for at least 28 days from the date of diagnosis

BBT wording

A non malignant tumour in the brain resulting in permanent deficit to the neurological system. Tumours or lesions in the pituitary gland are not covered

Medical evidence—10 year old boy

Claim form

- Diagnosis - Left sided porencephalic cyst
- Initial symptoms - extreme tiredness and sore head.
Right sided weakness. Pressure in back of eyes
- First consulted Dr - 01/08/09
- Investigations - CT/MRI
- Treatment - Trial of Pizotifen for headaches and
due to start another treatment to control seizures
- Previous symptoms? - No
- Family History? - No

NOTES

[illegible]

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BBT wording

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Medical evidence—10 year old boy

GP records

- 17/8/09 - Past 4 months has become lethargic. Previously sporty. Can finish game but takes to bed after. Performance much reduced. Blood tests needed.
- 2/12/09 - Pain behind eyes this morning. Minor hoarse throat. Apyrexial. Normal bloods. Possible mild chronic fatigue.

Hospital letters

- 13/1/08 - Temperature, No rash. Sore head. Lethargic. Possible viral URTI.
- 15/09/09 - Mum concerned due to FH of lymphoma and leukaemia in young people.
- 15/10/09 - 12 month history of lethargy

NOTES

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BBT wording

A non malignant tumour in the brain resulting in permanent deficit to the neurological system. Tumours or lesions in the pituitary gland are not covered

Medical evidence—10 year old boy

Hospital letters continued

- 09/04/10 - CT scan confirmed encephalic cyst. Has been investigated extensively for CFS but bloods negative. Impression of normal neurology with very subtle upper motor neuron signs in Rt lower limb. Headaches do not sound sinister. Refer Ophthalmology for fundoscopy, review images and request EEG to rule out epilepsy.
- 10/05/10 - Admitted with lethargy and headaches
- 22/07/10 - Headaches ongoing for almost a year
- 27/07/10 - Diagnosis likely chronic fatigue. Reasonable control over headaches. Weakness in right leg every so often. Mum says noticeable when unfit and first noted about 2 years ago.

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BBT wording

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Medical evidence—10 year old boy

Consultant report to insurer

- Diagnosis - L sided porencephalic cyst
- Date of diagnosis - 20/05/10
- Likely to have arisen as a pre or peri natal vascular insult
- Nil FH in respect of same condition
- Subtle evidence only of increased tone in R lower limb on clinical exam. Likely to be permanent as cystic lesion is stable/static in nature
- Takes pizotifen for headaches
- There will be a slight increase in lifetime seizure risk. No evidence of seizures at present.

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Medical evidence—10 year old boy

Further report from consultant to insurer

- Likely time of origin of cyst is the middle to late stages of the intrauterine period
- The lesion would explain the subtle motor signs which were highlighted at the outpatient clinic
- He is suffering from headaches and lethargy but continues daily activities normal for a 10 year old boy
- However, not doing as much physical activity as he was able to prior to his symptoms of tiredness and lethargy
- Tiredness and lethargy likely to be related to CFS rather than the cyst
- Blood tests all normal

NOTES

[illegible]

Child CI—Case Study 2 - DOB 6/9/2006

Policy 1

Policy definition - Policy Start Date 05/05/2011

For Children's Benefit to be payable, the child covered must be diagnosed with, or undergo surgery for, a Relevant Critical Illness or die during the period of cover. The period of cover will start from when the child covered is 30 days old and end on their 18th birthday, or 21st birthday if they are in full time education.

The Relevant Critical Illnesses under which Children's Benefit may be payable are (i) those Critical Illnesses defined in Condition 4(c) above but excluding Total Permanent Disability and (ii) the conditions listed in clause 4A(c) above (Additional Critical Illness Benefit).

- We will not pay the Children's benefit if the Relevant Critical Illness giving rise to a claim for Children's Benefit resulted from a condition existing before the child became covered by the Plan.

Policy 2

Policy definition - Policy Start Date 6/11/2006

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Medical evidence

B Presented on 11/9/2015 with longstanding symptoms of anorexia and poor weight gain going back two years. More acutely his presentation involved headaches, visual impairment and vomiting.

The exact diagnosis is Pilomyxoid Astrocytoma of the suprasellar region (WHO Grade 2) and was made following surgical excision and histopathology assessment on 12/7/2015.

James has a cancer predisposition syndrome known as Neurofibromatosis Type I and this predisposes him to having brain tumours.

This was not a congenital disorder however he has a predisposition syndrome.

CMO — I can confirm this child has an inherited condition but that this brain tumour is not congenital

NOTES

[illegible]

Child CI—Case Study 2

Policy 3

Policy definition - Policy Start Date 05/05/2014

Children's critical illness benefit

We'll pay this benefit if a **child** is diagnosed with, or undergoes surgery for, a **children's critical illness** during the

policy term and survives for at least 10 days.

- Definition of '**children's critical illness**'
-

An illness or condition suffered by the **child**, as detailed in the policy document, which the **child** survives for at least 10 days after meeting the relevant **critical illness** definition.

The illness or condition must not have been present at birth (whether diagnosed or not), and the symptoms must not have started before the start of the policy or before the **child** was covered by the policy.

ABI CI SOBP 2014

Cancer – *excluding less advanced cases*

Any malignant tumour positively diagnosed with histological confirmation and characterised by the uncontrolled growth of malignant cells and invasion of tissue.

The term malignant tumour includes leukaemia, sarcoma and lymphoma except cutaneous lymphoma (lymphoma confined to the skin).

For the above definition, the following are not covered:

- ☐ All cancers which are histologically classified as any of the following:
 - pre-malignant;
 - non-invasive;
 - cancer in situ;
 - having borderline malignancy; or
 - having low malignant potential;
- ☐ All tumours of the prostate unless histologically classified as having a Gleason score of 7 or above or having progressed to at least TNM classification T2bN0M0.
- ☐ Chronic lymphocytic leukaemia unless histologically classified as having progressed to at least Binet Stage A.
- ☐ Any skin cancer (including cutaneous lymphoma) other than malignant melanoma that has been histologically classified as having caused invasion beyond the epidermis (outer layer of skin).

Insurers are free to omit any of the model exclusions and may include additional exclusions.

Child cover – *pre-existing medical conditions*

A claim will not be covered for children's critical illness cover if:

- The child's condition was present at birth;
- The symptoms first arose before the child was covered; or
- The child dies within [28 days] of meeting our definition of the critical illness.

NOTES

[illegible]

Kat Ince—RGA Senior Pricing Analyst

Child CI Pricing considerations

- Getting the balance right between Child CI cover and the price paid for it
- Pricing accurately to reflect the claims that we expect to be paid under the Child CI offering
- Charging the right people across the book of policies for a Child CI policy

NOTES

[illegible]

IP Case Study (A)

RCD: 01/07/2009	EXP: 23/12/2048	Definition: OWN	Occupation: Service Point Officer (Sedentary role)	Age: 36
DFA: 02/06/2014	Cause of claim: Anxiety / Depression	DP: 26 weeks	S/A £7,625.95	GIP or IND: Group

Case Summary and questions:

The trigger for this ladies absence was Anxiety and Depression brought on by personal events. She was prescribed medication and placed on a NHS waiting list for therapy. At the start of her absence Occupational Health felt that a return to work should be possible within three months.

There were some obstacles in claims management from the beginning, for example, her husband had requested that all contact from her employer be via him. The claims assessor's initial attempts to contact her via telephone were unsuccessful as she did not answer the calls.

Her Occupational Health department was finally able to speak with her directly, after their second review they determined she was suffering severe symptoms and therefore a return to work within three months was now unlikely.

The claim form call went ahead via telephone and after a thorough conversation with the lady it came to light there were some work place stressors contributing to this ladies absence.

What would your next steps be?

- A) Offer claimant therapy from a provider
- B) Liaise with her employer to see if they were aware of any work place concern
- C) Encourage claimant to engage with work about these issues

We were able to offer her therapy during the telephone claim form call, but she declined this on the basis she would rather wait for the therapy she was already referred for (NHS).

What can you do when someone declines help?

We began to liaise with her employer specifically about any work concerns.

Medical evidence was received and it was found to be valid. Symptoms reported included not being able to leave the house and panic attacks. It was recommended she have therapy before attempting any return to work. The claim was accepted.

When would you look to next review this claim?

We chose to review this claim almost immediately as we wanted to monitor her progress with therapy. It emerged that she had told her therapist she was feeling better and a return to work was planned in three months. At this time the claimant expressed interest to reduce her hours upon her return.

What would you do at this point?

NOTES

[illegible]

Case Summary and questions:

We decided to monitor the claim closely to ensure we could do everything to support and encourage the return to work within three months. We queried the reason for reduction in hours (apparently a GP recommendation) and called the claimant for an update.

Three months passed and the claimant contacted us to advise she was not ready to return to work. A colleague had contacted her and as a result she now she felt too anxious. Her workplace was also rumoured to be closing down and she would be expected to move elsewhere.

What would you do at this point?

It seemed the trigger for her reluctance to return to work was a colleague and so we decided to provide vocational support to help overcome issues relating to the work place. A referral was set up.

Another return to work date was set for three months' time. This time the claimant was adamant she wanted to return to work for four days a week only. She asked for Mondays off specifically. She requested for us to support the reduction in salary with proportionate benefit.

What would you do at this point?

We required medical evidence to support her decision to reduce her hours, and why specifically Mondays? We investigated this further via her GP. We also asked a vocational provider to gather some information in this regard. The case was referred to a clinical expert to provide their opinion on this situation.

A consultant report was received and was entirely supportive of the claimant's request to have Mondays off. It emerged that her employer could only agree to a Wednesday off and this was why the return to work didn't proceed.

What red flags could this raise?

The red flag here was that claimant was so specific in her request for a certain day off – were there lifestyle issues at play? Also, it was unusual that a Consultant would be so supportive of such a specific work place adjustment.

The claimant continued to receive vocational support via the provider, and we received word that she had told her employers that the provider had told her not to speak to them or her Occupational Health department.

More red flags?

This appeared highly unusual. It was later confirmed the provider company had not given this advice.

The claimant then set another return to work date, this time she advised she wanted to return in the next month. This was the third return to work date she had agreed to. The provider company was able to investigate the exact barriers preventing her return (it had been very difficult to get the claimant to volunteer this information previously), and these barriers appeared to be her anxiety over her workplace closing down. They suggested she engaged with further therapy which we funded.

What could you do to help ensure this return to work does not fail again?

A psychological capacity evaluation was arranged. This determined that due to outcome scores of psychological testing as well as reported residual symptoms, she was not in a position to return to work. Unfortunately, shortly after the claimant's workplace closed down, however, her role remained and she was relocated. We decided to set a review for three months' time.

At the next review we called the claimant to see how she was doing, she advised she was not benefiting from the therapy we had funded for her. She said she was still keen to return to work but now she was unsure about her role as she had been moved to another location she was not fond of. It emerged she had recently travelled abroad to attend a friends' wedding (*remember, she said she couldn't leave the house?*), her symptoms seemed improved but evidence remained supportive.

What would you do at this point?

NOTES

[illegible]

Case Summary and questions:

It was decided that given the money spent on provider support, assessments and general time spent on liaising with her employer. We would suspend our efforts and review again in around 3-4 months.

In the following months a further two return to work dates were proposed by the claimant. We encouraged her Occupational Health department to re-open their file and attempt to manage this and took somewhat of a back seat approach to see if the return to work actually happened. Finally, we were in a position where the return to work was looking a little more certain. Specific adjustments had been requested by the claimant and most of these had been agreed by her employer, who were extremely flexible.

The remaining issue was that the claimant wanted us to support her reduction in hours with benefit on a permanent basis. The question was raised, if she can work X amount of hours why not her usual hours? If we did not agree claimants request it was highly likely her return to work would fail.

What would you do to ensure a return to work while still ensuring we can be cost effective in our assessment?

We discussed the case as a team and it was decided we would offer the claimant proportionate benefit to support her reduction in hours for one year only. Medical evidence was now questionable in terms of how supportive of her absence it actually was, recent correspondence from her Consultant was keen to agree her preferred working hours but with no real explanation as to how this would benefit her medically.

We would therefore, make it clear in our communications with her that we did not feel evidence supported her reduction and we were proposing this offer as a good will gesture. The offer was accepted and return to work successful and sustained.

This is a great example of short term loss for long term gain – yes a lot of money was spent, however, eventually, a return to work was achieved and was successful. If this claim had been left without attention, it's unlikely she would have ever returned. Given her young age and the ongoing red flags, it was seen to be a successful outcome despite expenditure.

Key outcomes/claims management:

- Phone call communication – frequent
- Utilise clinical expert advice and recommendation where available
- Vocational support and treatment provided
- Strong relationship with her employers/OH and consistent communication
- In depth medical evidence requested
- Psychological assessments to determine validity and return to work capabilities
- Team discussions
- "TCF" decisions and a gesture of goodwill offer agreed

NOTES

[illegible]

IP Case Study (B)

RCD: 01/07/2009	EXP: 23/07/2054	Definition: OWN	Occupation: Financial Guidance Advisor (part time) (sedentary)	Age: 27
DFA: 07/08/2013	Cause of claim: Back Pain / Low Mood	DP: 26 weeks	S/A £4,863.15	GIP or IND: Group

Case Summary and questions:

This young lady became unwell in 2013 due to back pain. This occurred during pregnancy and remained after. She also reported feeling very low due to the pain. The claim was very late notified to us making it difficult to assess. She was reportedly open to trying out other roles that allowed her to move around more freely. She had been absent for over 6 months yet was only taking pain killers.

What are the red flags here?

Six months with no treatment other than pain killers seems a little peculiar. Also, the claimant's reluctance to return to her own role could indicate some work place issues we may not be aware of. For example, were there other issues aside from the physicality of the role?

What could we do to investigate these red flags?

We offered provider based treatment for both her musculoskeletal issues (physio) and her low mood (therapy) and she agreed. We questioned her as to any work place concerns. It emerged there had been some problems with stress at work before she became absent.

There was no diagnosis for her back pain but medical evidence was supportive for both conditions. The claim was accepted.

She discontinued her psychological therapy stating it was not beneficial. She also discontinued her physiotherapy.

When would you next review this claim?

We decided to review the claim in three months. We reviewed with a medical update and found evidence was very brief. The claimant had barely visited her GP and the GP report highlighted the lack of ongoing treatment. The claimant was contacted to discuss this and she advised she was planning a return to work.

What could you do at this point to encourage her return to work?

As we were concerned at the lack of medical evidence we strongly believed a return to work should be taking place, we remained in close contact with the claimant and her employers to monitor the situation. Unfortunately the return to work failed, the claimant stated she was still in too much pain and was too anxious to return. We had the problem that we did not hold medical evidence to support her ongoing absence, yet the claimant had reported a relapse in her symptoms.

What would you do at this point?

NOTES

[illegible]

Case Summary and questions:

A functional capacity assessment (FCA) was arranged. The FCA stated her physical capability was appropriate for a return to work, yet her psychological symptoms could prevent a return. Although, these symptoms were of course self-reported. We still had concerns regarding validity despite the supporting FCA.

What would you do at this point?

Given the low sum assured, private investigators were not cost effective. However, we decided to perform desk based research ourselves. A large social media presence was found. The claimant has a makeup beauty blog as well as a youtube account and multiple social media pages. On one of her websites she talked about hating her day job and wanting to pursue a career as a makeup artist. She said that her day job had given her depression.

What should we be wary of at this point?

We were cautious of claimants make up "hobby" and needed to know if this was a paid job. We also needed to know how active she was in following this hobby. Also, the fact her motivation to return to her own job was so low was a concern. An interview/assessment was arranged with a provider company to find out more information about this claimant's activities as well as gain further information on the degree of her psychological ill health. Despite our concerns, the assessment was supportive of her continued absence and further NHS therapy was recommended. A review of 3-4 months was arranged.

At the next review, evidence received from her GP was extremely limited again. We phoned her to discuss this and asked her why she had not been to the GP again. She revealed she was in fact pregnant again and had been concentrating on this aspect of her health. She said her back pain was worse now because of the new pregnancy.

What would you do now?

Pregnancy does complicate claims assessment as maternity leave prevents a return to work for a significant period. As we had her social media accounts saved, we looked at these again for an update. There was a significant youtube video of the claimant walking around London, she was able to walk around in crowded places without any visible discomfort or pain displayed. The findings were recorded and discussed with the wider team.

Despite ongoing concerns and lack of evidence, the claimant went on maternity leave which meant we were able to suspend her benefit anyway. However, when she returns from maternity leave we will have to review her claim again. When this happens we will be able to investigate her on social media thoroughly to see if she has been active during her maternity leave. We will also be able to confront her with any concerns we may have and will need to be mindful she has a young child and so child care could factor in to her reluctance to return to work.

Key outcomes/ claims management:

- Conversations with claimant – approaching her regarding limited medical evidence and questioning this
- Desk based research – cost effective and useful in validity concerns
- Frequent medical reviews
- Assessments with providers – FCA / PCA
- Two conditions mean return to work barriers x 2 – identify which is the main barrier to a return to work and focus on this
- Clinical team input and advice
- A low sum assured can be restrictive but doesn't entirely prevent thorough investigation

NOTES

[illegible]

IP Case Study (C)

RCD: December 2005	EXP: 04/10/2031	Definition: Own occupation	Occupation: Operational Director of film production company.	DOB: 04/10/1971
DFA: 09/12/2009	Cause of claim: Depression	DP: 26 weeks	S/A £24,000	GIP or IND: Individual

Case Summary and questions:

A lady in her late 30's, working as an Operational Director in a company that she helped to set up. Role included duties revolving around HR/IT Facilities/Health & Safety. Partner also worked at the company. Company 9 years old.

Her annual benefit was £2000 pm, with an expiry age of 60.

A claim was submitted in March 2010, unable to work since December 2009 due to depression.

Two causes to the depression. Was expecting a child, but in July 2009, at 39 weeks, the baby stopped moving and she gave birth to a still born baby. Prior to this, and when she returned after 6 weeks of bereavement leave, 3 senior executives were bullying her at work, she claimed they had issues that a women was doing the role.

The claim form indicated that the claimant was taking legal proceedings against the firm and looking for compensation. Was taking medication of 20mg Citalopram.

What are your concerns at this stage?

- 2 separate issues, bereavement and work related issues
- Legal proceedings will delay claimant going back to work until settled

What would your next steps be?

- A) Admit the claim
- B) Arrange a home visit
- C) Obtain GP information
- D) Liaise with the employer to understand the work place issues

NOTES

[illegible]

Case Summary and questions:

We attempted b), c) & d)

We arranged a home visit with an expert in bereavement counselling. The claimant engaged well and wanted to work through the grief related issues, regain her self esteem/confidence. Claimant had been referred to a psychiatrist. Employers had made her and her partner redundant, no job to return too. Return to alternative work was unlikely until the bullying/litigation resolved. Legal proceedings commencing once SSP ceases. Was having destructive thoughts, but not suicidal.

Obtained a GP report and notes from the start of the pregnancy, with the view that there may be references to bullying prior to the birth of the baby and the issues ongoing afterwards. The report would give us the GP's thoughts on current capabilities, functioning and prognosis for a RTW. The report contained little info. Confirmed off work with anxiety and depression and referred to psychiatrist. Very scant notes.

Tried to get an employer form completed, but claimant was reluctant. Provided financial evidence to support claim financially.

What would you do at this point?

We agreed to admit the claim, but at the same time obtain a copy of the psychiatrist report to help manage the claim. The home visitor offered to help case manage the claim, continued telephone contact to build up a trust. Claimant advised that they got married and became pregnant again, but miscarried early in the pregnancy. Gynae issues were identified and was seeking fertility treatment. Legal proceedings had been started.

When would you next review claim? *Discuss how often you should review/how contact made (phone or letter)*

We decided to stay close to the claimant, keep the trust that we had built up. Phone calls every couple of months revealed she was taking no medication since her last miscarriage, as she wanted to become pregnant again and the medication may be harmful. Therapist was encouraging her to do some voluntary work. Still seeing psychiatrist. A friend had asked her to come into her school and listen to children read. Litigation date set. Until this taken place, no point trying to encourage paid work. Litigation took place and a settlement agreed. Claimant received 2 large sums of money, one for losing her job, one for injury to her feelings. In same call claimant announced she was 3 months pregnant.

What are you thinking at this point?

- Legal action sorted should mean a return to work
- She has received a large sum of money, she doesn't need to work
- She is pregnant and worried she will lose baby
- This claimant is not motivated to RTW
- Is she still depressed now that settlement is sorted

What would you do next?

We thought - Not going to achieve a return to work while pregnant, but need to encourage her to think about a return to work once baby is born and she had chance to adjust to family life. Decided to obtain a report from her psychiatrist after the baby was born and ask questions around return to work plans.

Report revealed that the birth triggered bad memories/self-harm thoughts. Husband got job in Manchester, he away in week, she is sole carer for baby. Health visitor very supportive and helping with post natal depression. Not on any meds.

Do we have enough medical evidence on this claim?

It was felt we needed a more independent view. Arranged a visit at home to carry out a vocational psychological assessment to try and get a better feel of the situation and what claimant was thinking, obstacles to return to work.

NOTES

[illegible]

Case Summary and questions:

- Obstacles were anxious over leaving son with anyone, including husband
- Socially anxious, struggled mixing with people
- Lost confidence
- Not sure where her home is going to be, thinking of moving to Manchester, but wants son to go to school in London.

But claimant stated she wanted to work.

Would you help the claimant and how?

We arranged work focused intervention, which included work focused coaching, develop skills and coping strategies, career guidance. Wants to be a primary school teacher.

First session arranged, but then we were advised claimant pregnant again. She decided she wants to wait starting work until 2nd baby 6 months old.

What is keeping this claimant off work now?

Illness

Children

Lifestyle choice

We felt it was the children and a lifestyle choice. Illness still there, but no longer primary issue.

We decided to obtain GP notes. GP states she is still depressed and seeing a counsellor regularly. No support network, panic attacks.

Speaking direct to the claimant, she stated she is moving back to London for son to go to school there. Then she will be able to look for work.

We agreed to work with her to career plan, think about childcare options.

When should we start the rehab work with her?

From previous events, we decided to wait until she actually moved back to London. Move got delayed due to house problems. 6 months later, still not moved.

We decided to look for a 'green shoot' to indicate a positive approach to study/employment.

Current status of this claim -

Phone call from claimant telling us that she moving to Barcelona as husband's job taking him there.

Father and father in law also live out there, so she will have support. Policy has a residency exclusion on it, claim stopped until she returns to UK. At that point, we will need independent medical evidence to restart claim.

Key outcomes/claims management

- Phone call communication – frequent
- Red flags – bio psychosocial issues
- Assessments with providers – picking the right person for the job
- Having no job to return to makes it harder to get a claimant back into work – need to work with the claimant
- Two triggers to depression, mean return to work barriers x 2
- Litigation/compensation battles make claim management difficult
- High sum assured reduces incentive to return to work.

NOTES

[illegible]

IP Case Study (D)

RCD: 24.6.96	EXP: 17.1.2034	Definition: OWN or SUITED	Occupation: S/E Builder, previous experience cleaning manager	DOB: 17/01/1969
DFA: 16.2.2011	Cause of claim: Neck pain, visual symptoms, anxiety & depression, aneurysm. Previous sub-arachnoid haemorrhage in 1997 due to ruptured aneurysm.	DP: 4 weeks	S/A £8029 pa	GIP or IND: Individual

Case Summary and questions:

Background

Claim form initially confirmed incapacity to be neurological/cognitive symptoms secondary to SAH in 1997. The subsequent GPR advised the cause of incapacity to be Depression with dizziness and visual disturbance. In view of the combined neurological symptoms and Depression the policy holder was referred for a Neuro-psychiatrist appointment (which had a 4 month wait).

The policy holder was also under the pain clinic and the Rheumatologist suggested neck pain not sufficient itself to prevent working and visual tests also reported as normal.

During investigations into the neurological issues the policy holder underwent a CT scan where an aneurysm was found which was unsuitable for coiling. Claim was admitted on the basis that heavy manual work would likely be prevented with an untreated (unstable) aneurysm.

The plan as a Mental health exclusion '*No benefit shall be payable for any illness or disability directly or indirectly attributable to depression anxiety or any disorder without a demonstrable physical basis*'

At this stage, would you have any concerns with this claim and what action would you take next?

Concerns were that the claimant continued to work for many years with these symptoms but had only apparently ceased work on discovery of the new aneurysm.

Evidence suggested neck pain was not severe enough to prevent work and lack of objective evidence to support reported visual symptoms.

Could anxiety and depression be accounting for his symptoms? What would you do next?

Medical review undertaken and a report obtained from the treating consultant neurologist who confirmed a 1-2% risk of rupture. The claimant had declined any treatment so watch and wait approach adopted. The neurologist added that he felt it unlikely that reported symptoms which, included daily chronic headaches, were related to his current aneurysm but possibly due to anxiety and/or the previous subarachnoid hemorrhage.

What would you do at this point? – Could the exclusion be applied?

NOTES

[illegible]

Case Summary and questions:

Claim referred to CMO neurologist who agreed that due to the very low risk of rupture and with a stable aneurysm that the claimant should not have to refrain from heavy manual work. Also, no evidence to suggest that his symptoms were being caused by the aneurysm itself.

However, additional medical evidence was sought from the GP who suggested that visual disturbance and headaches were the main causes of the claim. After reviewing the further evidence, CMO neurologist concluded that there was no evidence to support his headaches being caused by the previous subarachnoid haemorrhage or current aneurysm. He did however advise that anxiety and depression could exacerbate his symptoms and concluded that, as a condition, headaches on their own should not prevent working.

What would you do now?

IME with a headache specialist arranged which supported the validity of the claim but remained inconclusive over the cause of the symptoms.

A copy of the IME was sent to the GP recommending a neuro-psychiatrist referral..

At the first review, the insurer wrote to the GP to follow up on the neuro-psych referral but the claimant had declined this. A referral for counselling had been made so an update on this was requested but was never received so the insurer actually approached the GP for copy notes.

These showed that he had not engaged with therapy and had been discharged as he had advised that he was moving to Malta to set up a business with his wife.

What action would you now take?

The insurer returned to the GP to obtain copies of all consultations for the past year to check details of all attendances the claimant had made to the surgery, particularly in relation to his headaches and there was reference that his headaches and depression had improved to the point that he could think about stopping medication. His aneurysm remained stable with no further treatment planned and again it was noted that he was leaving the UK for 3 months to spend time in Malta.

The insurer reviewed their residency clause but as Malta is part of the EU and the GP notes suggested he had returned to the UK after 3 months, they concluded that this ruling would be difficult to enforce.

Enquiries were then made in Malta by a claims investigation provider, which showed that the claimant had visited and purchased a vehicle in Gozo, but due to the very small island community, further enquiries would likely be compromised. We knew that the claimant had returned to the UK by this stage so another report was obtained from the treating consultant neurologist who confirmed;

'Symptoms he described are likely to continue including headache, fatigue & possible migraine auras. No doubt that these symptoms combined with anxiety affect his activities but not ADL's -many people with aneurysms continue to work in occupations such as claimant's but certainly possible that the level of his anxiety akin to PTSD may prevent him from working'.

What would you do now?

CMO view obtained which stated that the basis of the claim was subjective reporting of his headaches and anxiety/depression but due to the cerebral history, there was sufficient evidence for the claim to continue despite the consultant's comments that he could work.

Legal view obtained on the mental health exclusion, which specifically states *'No benefit shall be payable for any illness or disability directly or indirectly attributable to depression anxiety or any disorder without a demonstrable physical basis'*.

As there was a physical basis, i.e. the aneurysm, for his symptoms of anxiety we conceded that the exclusion could not be enforced.

How would you proceed?

NOTES

[illegible]

Case Summary and questions:

Full file review and social media search undertaken which revealed a photo of the claimant on his wife's social media account sitting in the driver's seat of a white, sign written vehicle with part of a mobile phone number visible. The insurer was able to match the number to that provided by the claimant on a continuation form and 2 days of initial surveillance were obtained. The surveillance showed the claimant loading tools into a van, sign written with the name of a building and maintenance company. Visiting a builder's merchant, sawing and loading wood into the van.

What would you do now?

Further 2 days of surveillance organized where the claimant was seen at a residential address for both days, regularly leaving and entering the property with the impression given that he was carrying out work inside and again seen at a timber merchant.

What action would you now take?

A continuation form was issued to the claimant at this point to verify his ongoing symptoms and he confirmed that he remained unable to work but required an urgent brain scan due to his aneurysm possibly increasing in size.

Additional evidence relating to the aneurysm was received but following a scan, it was confirmed no change had taken place and it remained stable.

What would you do now?

Cost benefit analysis undertaken to consider further surveillance. Total costs already exceeded £6K with a remaining liability of circa £144K until expiry.

As surveillance so far suggested that claimant was potentially carrying out working activity, 3rd period of surveillance to coincide with a claims visitor was arranged to compare and contrast his reported symptoms and activity to what he actually reported.

Footage was of poor quality due to the location but suggested that the claimant was in a garden of a property with a cement mixer operating, apparently laying a breezeblock.

The claims visitor reported that the claimant continued to experience restrictions due to headaches and fatigue, cognitive and visual disturbances and anxiety around the remaining aneurysm. He went on to report that he was unable to carry out any heavy tasks, would be unable to work due to inability to sustain any activity for any period of time and would be concerned around using a ladder or power tools.

A legal view was obtained due to concerns over the quality of the surveillance and it was felt that whilst highly suggestive that the claimant was working, it was not of sufficient quality to prove that he was working and potentially would provide him with opportunity to deny the activity.

How would you proceed with this claim? Would you continue with further surveillance or accept that the claim should continue?

A further period of surveillance was recommended but was not considered cost effective. From a claims perspective it was felt that we had sufficient evidence to suggest that this man was at least, significantly more active than he had recently reported to the claims visitor and that potentially, he was carrying out working activity.

The insurer decided to confront the claimant and this was carried out over a recorded telephone line by the assessor. He advised the claimant of our suspicions without confirming the exact nature of the evidence that we had, leading the claimant to confirm that he carried out some work.

The claimant then called back the next day, to provide more details around the work that he had carried out for the previous year or so, for which he had received payment. After consideration of the extensive admissions that he made, it was felt that he had deliberately misrepresented his position to the insurer, the claim was ceased and his IP and associated WOP policies were cancelled completely.

NOTES

[illegible]

Case Summary and questions:

What would you do now? Would you refund premiums or refer for further investigation?

The insurer initially wanted to refund premiums but after consideration of what we felt were deliberate actions; the insurer declined to refund premiums and reserved their right to refer to IFED.

*Due to the relatively low amounts involved it was felt that IFED were unlikely to carry out an investigation.

NOTES

[illegible]

Carcinoid—case study 1

Policy definition

Cancer. A malignant tumour characterised by the uncontrolled growth and spread of malignant cells and invasion of tissue. The term cancer includes Leukaemia and Hodgkin's disease but the following are excluded :

- All tumours which are histologically described as pre-malignant, as non-invasive or as cancer in situ.
- All forms of lymphoma in the presence of any Human Immunodeficiency Virus.
- Kaposi's sarcoma in the presence of any Human Immunodeficiency Virus.
- Any skin cancer other than malignant melanoma.

Medical evidence

Consultant Oncologist to Insurer

- **Diagnosis -** Typical lung carcinoid. Right middle lobe. Tumourlet in Right lower lobe. My recommendation was surgical resection.
- **Staging -** T1a N0 PL0 R0
- **Was the disease completely localised? -** Yes
- **Was there invasion of adjacent tissues? -** No
- **Were regional lymph nodes involved? -** No
- **Were there distant metastases? -** Cannot say either yes or no.
Tumourlet in Right lower lobe, the significance of which in typical carcinoid is uncertain.
- **Any other comments? -** Current guidelines clearly state that all carcinoids should be treated the same as non-small cell lung cancer.

NOTES

[illegible]

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Medical evidence

Histology report

- **Frozen section -** Epithelial tumour favouring carcinoid. No atypical features.
- **Microscopic -** Paraffin sections confirm the above report, with additional sections showing a carcinoid without atypical features. There is no necrosis and mitotic figures are not obviously apparent.
- **Conclusion -** Right middle lobe, typical carcinoid.
pT1a N0 PL0 R0

Supplementary report

- The crushed epithelial cells stain diffusely for CD56, with only very occasional cell staining with Ki-67. Staining for MNF116 is negative. On this basis, this nodule is most likely a tumourlet.

NOTES

[illegible]

Carcinoid—Case Study 2

Policy definition

Cancer—any malignant tumour positively diagnosed with histological confirmation and characterised by the uncontrollable growth of malignant cells and invasion of tissue. The term malignant tumour includes leukaemia, sarcoma and lymphoma except cutaneous lymphoma (lymphoma confined to the skin).

For the above definition, the following are not covered—all cancers which are histologically classified as any of the following:

- Pre-malignant
- Non-invasive
- Cancer in situ
- Having either borderline malignancy or
- Having low malignant potential
- All tumours of the prostate unless histologically classified as having a Gleason score greater than 6 or having progressed to at least clinical TNM classification T2N0M0
- Chronic lymphocytic leukaemia unless histologically classified as having progressed to at least Binet Stage A
- Any skin cancer (including cutaneous lymphoma) other than malignant melanoma that has been histologically classified as having caused invasion beyond the epidermis (outer layer of the skin)

Medical evidence

Claim form

- **Nature of illness** - Neuroendocrine carcinoid tumour in the Appendix.
- **Which illness are you claiming against?** - Cancer
- **First symptoms** - Agonising abdominal pain. Admitted to hospital. Surgery performed to remove appendix when tumour was discovered.

NOTES

[illegible]

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Medical evidence

Letter from consultant to claimant

I am writing to confirm that your case was discussed at the regional multidisciplinary meeting for neuroendocrine tumours and the outcome is completely in keeping with the advice you have been given before.

The pathology report shows a small 7mm neuroendocrine tumour confined entirely to the appendix. This has been completely excised and no further investigation or follow up is required.

These lesions although not particularly common have no further adverse outcome regarding your health. We can safely discharge you, and this should have no further implications with regards to life insurance or getting a mortgage.

NOTES

[illegible]

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Medical evidence

Consultant report to Insurer

- | | | |
|--|---|--|
| • What is the exact diagnosis? | - | 7mm incidental well differentiated neuroendocrine tumour (carcinoid). Found in appendix specimen following admission for acute appendicitis. |
| • What stage did the disease reach? | - | <u>This is not a cancer</u> (his emphasis) |
| • What treatment is still to be given? | - | None |
| • Are they in remission? | - | N/A |

NOTES

[illegible]

